

ST. JUDE METROPOLITAN HOSPITAL - CLINICAL RECORD

CONFIDENTIAL MEDICAL INTELLIGENCE REPORT

EMERGENCY MEDICINE & TOXICOLOGY REPORT

Patient Name: Elena Rostova

Age: 29 | Gender: Female | MRN: #ER-409182

Encounter Date: 2026-08-25 04:15 AM

Attending: Dr. Marcus Vance, MD (Emergency Medicine)

CHIEF COMPLAINT & PRESENTATION:

Patient brought in via EMS following acute intentional ingestion of unknown quantity of Acetaminophen and alcohol approximately 4 hours prior. Presents with diaphoresis, epigastric pain, and persistent emesis.

CRITICAL VITALS:

BP: 94/60 mmHg | HR: 112 bpm (Sinus Tachycardia) | RR: 22 bpm | Temp: 37.4 C | SpO2: 95% on room air

URGENT LAB BIOMARKERS:

- Serum Acetaminophen Level: 185 mcg/mL [CRITICAL HIGH - Above Rumack-Matthew treatment line]
- AST (SGOT): 420 U/L [CRITICAL HIGH] (Ref: 10 - 40 U/L)
- ALT (SGPT): 380 U/L [CRITICAL HIGH] (Ref: 7 - 56 U/L)
- Total Bilirubin: 2.1 mg/dL [HIGH] (Ref: 0.2 - 1.2 mg/dL)
- Serum Lactate: 3.8 mmol/L [HIGH] (Ref: 0.5 - 2.0 mmol/L)
- Blood Alcohol Concentration (BAC): 0.14 g/dL

ASSESSMENT & IMMEDIATE INTERVENTIONS:

1. Acute Acetaminophen Toxicity with early hepatic injury.
2. High risk of fulminant acute liver failure.

RECOMMENDED ACTION PLAN:

1. Priority 1 (Triage Nurse): Immediately initiate IV N-acetylcysteine (NAC) 21-hour infusion protocol.
2. Priority 2 (Attending Physician): Urgent Medical Toxicology and Hepatology consultation for liver transplant backup alert.
3. Priority 3 (Laboratory): Repeat LFTs, Coagulation Panel (PT/INR), and venous blood gas every 4 hours.